

Hoarseness - Female (Adult)

Algorithm 1190 | Adult (18 years and older) | Gender at birth: Female

Definition

- Safety-first assessment of hoarseness or suspected laryngitis in children and adults, excluding acute upper-airway obstruction and identifying infection, immunocompromise, and possible malignancy red flags.
- Adult patient.
- Female.

Initial assessment

1. [OPEN_TEXT] Record age, onset and duration, recent choking or inhaled foreign body, infection or intubation, recurrent episodes, smoking or vaping, occupational voice use, and previous head, neck, or chest cancer.
2. [YES_NO] Is there noisy breathing or stridor, chest or neck pulling in, pauses in breathing, blue colour, inability to speak or drink, drooling or inability to swallow saliva, tripod posture, severe drowsiness, or rapid worsening?
3. [YES_NO] Is there severe throat pain, neck swelling, high fever or severe illness, blood when coughing, dehydration, immune suppression, or recent chemotherapy?
4. [YES_NO] Is hoarseness persistent or unexplained, especially in an adult aged 45 or older, or accompanied by a neck lump, progressive swallowing pain, unexplained weight loss, or smoking history?

Triage and redirect questions

1. Redirect → Anaphylaxis

Are sudden throat or tongue swelling, wheeze, faintness, confusion, or collapse the main concern?

Sudden systemic allergic-reaction features require the Anaphylaxis pathway.

1. Disposition level 100

Is there difficulty breathing, stridor at rest, chest or neck pulling in, pauses in breathing, blue/grey colour, inability to speak or drink, drooling or inability to swallow saliva, tripod posture, reduced responsiveness, or rapid worsening, especially in a child?

These are possible upper-airway-obstruction signs. Children have smaller airways and can deteriorate rapidly; drooling and inability to swallow saliva can indicate a serious airway infection or obstruction.

1. Disposition level 70

Without an emergency airway feature, is there severe throat pain, swallowing difficulty, neck swelling, high fever or severe illness, dehydration, coughing blood, immune suppression, recent chemotherapy, or sudden hoarseness after choking or possible inhaled foreign body?

These features require prompt direct assessment for serious infection, deep-neck disease, foreign body, bleeding, or complications in an immunocompromised patient. The exact Qatar service and timeframe are governance-required.

1. Disposition level 50

Is hoarseness persistent, recurrent, or unexplained, particularly in a person aged 45 or older, or associated with a neck lump, progressive swallowing pain, unexplained weight loss, smoking or vaping, or previous head, neck, or chest cancer?

Persistent or unexplained hoarseness needs examination. NICE advises consideration of a suspected-cancer pathway for persistent unexplained hoarseness or an unexplained neck lump in people aged 45 and over; Qatar referral criteria require local governance.

1. Disposition level 15

Is this recent mild hoarseness in a patient who is otherwise well, drinking normally, breathing quietly, with no drooling, stridor, severe pain, immune suppression, cancer red flag, or other feature above?

NHS.UK guidance: laryngitis usually resolves within 1-2 weeks, often worse in the first 3 days.

Advice

Emergency upper-airway precautions

- Call Qatar 999 now. Keep the patient calm and in the position that makes breathing easiest; do not force them to lie down, inspect the throat, or give food, drink, tablets, or steam. Do not allow self-driving. Follow the call-handler's airway and resuscitation instructions. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: increasing stridor, chest or neck recession, drooling, blue/grey colour, exhaustion, or reduced responsiveness.

Prompt in-person hoarseness assessment

- Use the Qatar governance-approved in-person service and timeframe. Keep the patient calm and do not attempt throat examination or use steam inhalation. If swallowing is comfortable, offer normal cool fluids; escalate immediately for stridor, drooling, breathing difficulty, inability to drink, blue colour, or drowsiness.
- Call Back If: stridor, drooling, breathing difficulty, inability to swallow, blue/grey colour, or drowsiness; worsening fever, neck swelling, dehydration, or coughing blood.

Persistent-hoarseness assessment

- Use the Qatar governance-approved primary-care or ENT pathway; this UAT protocol does not assign a definitive cancer diagnosis or exact referral timeframe. Persistent unexplained hoarseness, a neck lump, progressive swallowing symptoms, weight loss, smoking history, or previous cancer requires documented clinical examination and referral consideration.

- Call Back If: new breathing or swallowing difficulty; neck swelling, blood, weight loss, or worsening symptoms.

Home care for strictly low-risk hoarseness

- Rest the voice, speak gently rather than whispering or shouting, take normal fluids, and avoid smoking, vaping, smoke, dust, and excess alcohol. Do not use bowls or kettles of steam because of burn risk. Confirm medicines and age restrictions with a pharmacist.
- Call Back If: stridor, drooling, breathing or swallowing difficulty, fever with severe illness, or dehydration; persistent, recurrent, or unexplained symptoms, neck lump, blood, or weight loss.

First aid

- Call Qatar 999 now. Keep the patient calm and in the position that makes breathing easiest; do not force them to lie down, inspect the throat, or give food, drink, tablets, or steam. Do not allow self-driving. Follow the call-handler's airway and resuscitation instructions. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Laryngitis", <https://www.nhs.uk/conditions/laryngitis/> (page last reviewed 17 January 2024)
- Cambridge University Hospitals NHS Foundation Trust, "Children's croup and stridor", <https://www.cuh.nhs.uk/patient-information/childrens-croup-and-stridor/> (accessed 25 July 2026)
- NICE NG12, "Suspected cancer: recognition and referral", <https://www.nice.org.uk/guidance/ng12/chapter/recommendations-organised-by-site-of-cancer> (accessed 25 July 2026)
- Qatar Ministry of Public Health, "Healthcare Services in Qatar", <https://sportandhealth.moph.gov.qa/EN/faninfo/Pages/HealthcareServicesInQatar.aspx> (999 ambulance access; accessed 25 July 2026)

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

Manifest hash: b0a1e119c93c7d7a9799c102774ce126463cb11b02c8f6da084c4f8403c3a280 | Status: BLOCKED_BY_RESEARCH_GAP